Cavendish Authority

Medication Governance Standard — Adult Behavioral-Health Inpatient Units

CA-MG-14 | Issued 15 January 2026 | Applicable to inpatient services

1. Purpose and application

1.1 This standard establishes the minimum medication-governance controls for adult behavioral-health inpatient units. Services must apply the controls with their physical-plant safety assessment and local emergency arrangements.

1.2 Medication information and records must be available only to people involved in care, operations, safety, or authorized oversight. Handoffs must use the minimum information needed for the receiving role.

2. Storage and access

2.1 Medication must be secured from patients and unauthorized staff. Access is limited to authorized personnel and must be accountable through an access record or equivalent auditable control.

2.2 Storage and access hardware must account for ligature risk and use tamper-resistant physical design. A service must not depend on wall-mounted medication cabinets where its physical-plant assessment excludes them.

2.3 Refrigerated medication must be held in access-restricted storage with temperature monitoring. A temperature excursion requires corrective action, assessment of product integrity, and a record.

3. Orders, review, and patient-specific supply

3.1 Administration requires a valid clinical authorization and verification of the intended patient, the current order, and relevant safety information. Patient-specific medication must be labeled for that patient and verified before administration.

3.2 A pharmacist must review medication orders within the service review window. If waiting for the next delivery or pharmacist review would create an unsafe clinical delay, a clinically authorized emergency or first-dose pathway may be used. The use and reason must be documented and pharmacist review obtained retrospectively.

3.3 A patient's medication must not be used for a different patient. Patient-specific supply must not be relabeled, shared, or carried forward between patients.

4. Contracted pharmacy delivery and contingency supply

4.1 Contracted-pharmacy deliveries require a documented receiving handoff. Authorized receiving staff must verify delivery contents against expected patient-specific and contingency items, place them securely, and record and escalate discrepancies.

4.2 Emergency or override stock is for a defined urgent clinical need only. It is not routine convenience stock or a substitute for ordinary supply. Each access must be authorized, documented, inventoried, and reconciled after use.

5. Controlled medication

5.1 Controlled medication requires stronger custody than ordinary medication, including secure restricted access, shift or access reconciliation, and a custody record.

5.2 A controlled-medication discrepancy requires prompt escalation, witnessed handling as applicable, investigation, and a record of the discrepancy and resolution.

6. Administration, exceptions, and transition

6.1 The administration record must document administrations, refusals, omissions, PRN indication and response, and urgent or emergency access.

6.2 Medication changes, discontinuations, returns or wastage, and unresolved issues must be documented and communicated at handoff. Supply that is no longer active must be reconciled and handled through the authorized return or wastage process.

6.3 A service must have an escalation pathway for discrepancies, delayed or unavailable doses, adverse reactions, and urgent medication changes. Immediate patient safety action, appropriate clinical notification, documentation, and follow-up are required.

Authority note: This standard establishes governance controls; it does not replace a prescriber's clinical judgment or service emergency procedures.